

How to Use This Resource

In lieu of a study guide on cultural competency and cultural humility, this free worksheet packet provides you with resources to review and self-study as well as worksheets and prompts to prepare for the exam.

How to use this worksheet packet:

1. Flip through this packet to see what is included.
2. Download the following resources from the links (8 total). Download resources from additional links you see in subsequent sections, like in Islamic practices, now or wait until you get to those sections.
3. Read through the Introduction and Models of Cultural Competence section. Complete the cultural competence worksheets in Freebie Packet 4.
4. Read the resources you downloaded. Fill out the 2 worksheets at the end of this packet with any pertinent information.
5. Continue reading the sections in this packet on specific culture and religious groups, which will prompt you to find out specific things about each religion or culture as it relates to food and dietary practices. Continue filling out the 2 worksheets at the end of this packet with information that you find.

Resources to Read and Review

Please review the following resources in addition to the information in this packet

- Fox, M. Global Food Practices, Cultural Competency, and Dietetics. *JAND*. 2015. <https://bit.ly/3CKJAUC>
- Amidor, T. Ask the Expert: Counseling Clients from Diverse Cultures. *Today's Dietitian*. 20:9, 10. 2018. <https://www.todaysdietitian.com/newarchives/0918p10.shtml>
- Palmer, S. Cultural Humility in Food & Nutrition. *Today's Dietitian*. 23:2, 24. 2021. <https://www.todaysdietitian.com/newarchives/0221p24.shtml>
- Masters C, Robinson D, Faulkner S, Patterson E, McIlraith T, Ansari A. Addressing Biases in Patient Care with The 5Rs of Cultural Humility, a Clinician Coaching Tool. *J Gen Intern Med*. 2019;34(4):627-630. doi:10.1007/s11606-018-4814-y. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC6445906/>
- Charles-Alexis, A. Cultural Competence in Nutrition and Dietetics: What We Need to Know. *Healthline*. April 9, 2021. <https://www.healthline.com/nutrition/cultural-competence-in-dietetics>

You should also be aware of the Inclusion, Diversity, Equity, and Access (IDEA) resources from the Academy of Nutrition and Dietetics. As CDR is the test-writing and accreditation arm of the Academy, there are sometimes questions related to Academy initiatives.

- The IDEA Hub. <https://www.eatrightpro.org/practice/practice-resources/diversity-and-inclusion>
- Advancing Equity: The Academy's Commitment to Supporting Inclusion, Diversity, Equity, and Access. *JAND*. 2022. 122:1, 159-165. [https://www.jandonline.org/article/S2212-2672\(21\)01466-0/fulltext](https://www.jandonline.org/article/S2212-2672(21)01466-0/fulltext)

Finally, you should be aware that there are cultural competence standards established by The Joint Commission. You can read about them here: <https://www.jointcommission.org/-/media/tjc/documents/resources/patient-safety-topics/health-equity/roadmapforhospitalsfinalversion727pdf.pdf>

If you have access to the book 'Cultural Food Practices' by the Diabetes Care and Education Dietetic Practice Group (edited by Goody M and Drago L) I would encourage you to focus on the introduction paragraphs for each section, and the end of each section that focuses on specific recommendations. The book can be purchased online through the Academy or other sellers, like Amazon, or may be available through your program or University library.

Introduction

Culture is the accumulation of a group's learned and shared behaviors in everyday life. Through your formal and informal interactions with clients you will see how culture defines eating occasions in certain ways.¹ It is paramount that the nutrition professional practices cultural competence to establish rapport with clients when providing nutrition education to help improve their health. **In this packet, you'll see information organized by 1) considerations for how you interact with clients and 2) dietary modifications common in a cultural group or religious group.**

Models of Cultural Competence

Specifically, the RDN should be familiar with the **Campinha-Bacote Model of Cultural Competence**² and implement the model in their nutrition practice.

Starting in the individual, this model states that cultural competence expands to the family and community level.

The 5-step process focuses on 1) awareness, 2) knowledge, 3) skill, 4) encounter, and 5) desire.

You can remember the 5 steps using the mnemonic "**ASKED**", in the sense of "Have I ASKED myself the right question"?

- **Awareness** involves challenging assumptions and asking client questions about their values, beliefs, and practices, resulting in acquisition of cultural knowledge.
- With **cultural knowledge**, the provider familiarizes themselves with variations in cultural impact and expands their **cultural skills** to collect key information to conduct effective assessments and decide on an intervention that takes into account preferred language, foods consumed or avoided (especially when feeling ill), and foods consumed or avoided for religious reasons.
- This **cultural encounter** is the best time to discuss nutrition.
- The final step is **cultural desire**, where the practitioners want to engage in cross-cultural care.

For the exam, know 5 steps and their definitions.

- 1.
- 2.
- 3.
- 4.
- 5.

Other cultural competence models to be aware of include The ETHNIC model of Culturally competent care, The LEARN model, The BATHE Model, The GREET Model, The Purnell Model, Explanatory Models Approach. These are covered in Part 2 of the Management Concepts Study Guide by the Study Smarter Method and have associated worksheets in Freebie Packet 4 at www.studysmartermethod.com/freebies.

Questions on the RD exam may revolve around cultural competence as it relates to nutrition counseling and client interactions and/or specific food restrictions or practices related to different cultures or religions. You should also be aware of population health trends by race/ethnicity groups, such as the higher prevalence of type 2 diabetes in the United States among non-white race/ethnicity groups.

Religious Dietary Food Practices

You can think about religious dietary food practices in several categories:

- **Food groups:** Do individuals following this religion abstain from certain food groups? *Ex: Seventh-Day Adventists typically follow a lacto-ovo-vegetarian diet.*
- **Food groups and holy days:** Do individuals following this religion abstain from certain food groups on certain occasions? *Ex. Catholics do not consume meat on Fridays during lent. Eastern Orthodox Christians abstain from alcohol, eggs, dairy, fish, meat, and olive oil during weekly fasts.*
- **Fasting:** Do individuals following this religion regularly fast?
- **Food preparation:** Does a religion require food to be prepared a certain way? *Examples: Kosher (Judaism), Halal (Islam), and special considerations for Brahmins, a class of Hinduism.*

Note: Rather than memorizing different aspects of each religion, I've identified key categories that are common between different religions. This re-organization is an example of *studying smarter*.

Food Groups

Several religious groups adopt vegetarian or modified vegetarian diets because their religion specifically prohibits consuming animal products or because certain tenets are interpreted that way. Some religions also prohibit alcohol, tobacco, and caffeine.

Hinduism and Buddhism do not prohibit eating meat, but many followers may follow vegetarian or lacto-vegetarian diets. In Hinduism, cows are sacred, so while eating beef is prohibited, dairy products are acceptable.³ Dietary practices vary by different forms of Buddhism, with some interpret one of the Buddhist teachings, which prohibits taking the life of any person or animal, that they should not consume eggs, poultry, fish, or meat.⁴ So, while some Buddhists may follow a lacto-vegetarian diet, others may consume meat and other animal products. Hindus and Buddhists may avoid pungent spices and vegetables, such as onions, garlic, scallions, chives, and leeks, as they are thought to hinder spiritual development or excite impure desires.^{3,4} Jainism is a religion of non-violence. In the practice of protecting all life, Jains do not eat eggs, fish, meat, poultry, or honey. They typically avoid root vegetables since the plant is killed upon harvest. They fast during holy days, breaking the fast only before sunset, and favor grains over green vegetables or raw vegetables, the latter of which have been seen as more alive than grains.

Seventh-day Adventists avoid fish, meat, poultry, alcohol, tobacco, and caffeinated beverages such as coffee. Since they do eat eggs and dairy (sparingly),³ they follow a *lacto-ovo-vegetarian diet*. The Church of Latter-Day Saints, also called the Mormon Church, has members abstain from alcohol, caffeine, and sometimes soda. They emphasize whole foods and eating foods that are in-season. The main principles surrounding Mormon dietary practices is not necessarily the foods allowed or not allowed, but the principle of moderation and appreciation, emphasizing portion control, minimizing food waste, and avoiding overindulgence. Mormons also consider fasting a show of self-control and humility, and commonly fast monthly.⁵

Rastafarians also follow tenants of “clean eating” with the rule of I-tal (or ital), meaning “eating natural and clean”, or a diet that is free from additives, chemicals, most meat, and helps them to stay healthy and connected spiritually to the Earth.⁶ I-tal emphasizes raw and natural foods, though lightly cooked meals are allowed. While some meats are allowed, it is not common, and pork, the meat of scavengers, and shellfish are not eaten. Salt, alcohol, milk, and coffee are avoided among some Rastafarians. Like with other religions, followers vary in how strictly they follow the dietary practices, with some following a strict vegetarian diet, while others are vegan or pescatarian.⁷ Rastafarians do not fast for holy days, and feasts for holy days will be strictly vegetarian or vegan.

Holy Days, Food, and Fasting

Many religions may abstain from certain food groups during holy days. For example, during Lent, Catholics are to not eat meat on Fridays, and instead commonly eat fish. They also fast for Ash Wednesday (the first day of Lent) and Good Friday (the Friday before Easter). These fasts involve 1 main meal (typically the evening meal) and 2 small meals that, together, are less than the main meal.⁸

Practicing Eastern Orthodox Christians will fast weekly on Wednesdays and Fridays, and when breaking their fasts they will not consume alcohol, eggs, dairy, fish, meat, or olive oil. Throughout the year, meat is prohibited on Wednesdays and Fridays, and there are four extended fasting periods before principal feasts: before Christmas (Lent in the East, Advent in the West), before Easter (Great Lent), after Pentecost (Fasting of the Apostles), and for 2 weeks prior to the feast of Assumption of Theotokos (August 15th).⁹ The following link provides practical information on fasting rules in the Orthodox church: <http://www.abbamoses.com/fasting.html>

You'll notice that dietary practices including a change in the typical foods eaten and fasting, are noted in the previous section, and for Islam and Judaism, in the coming sections.

Food Preparation

In Islam, certain foods are categorized as *halal* ("lawful, permitted"), *haram* ("harmful"), and *mashbooh* ("doubtful"). Note that these terms are not used exclusively for foods but are applied to other aspects of life. Religious food practices may vary among Muslims, with some consuming only halal foods, others consuming meats not considered halal (but typically not pork), and some limiting fish consumption to fish that have scales. Pork, products made from pork or non-Zabihah beef, blood and blood byproducts, and alcohol are some of the main haram foods. *Zabihah* means the animal was slaughtered according to Islamic dietary laws.

Kashrut (Dietary Laws) is a set of Jewish religious dietary laws that outlines what food is *kosher*, meaning fit for consumption. The dietary laws prohibit certain foods (pork, shellfish), the mixing of meat and milk, that meat and poultry be slaughtered according to Jewish dietary law (*shechita*) as well as laws about sustainability in agriculture.¹⁰ To determine if an animal is kosher, mammals must have split hooves and chew its cud (includes cows, sheep, goat, deer; excludes pigs, rabbits, horses, camels, wild game, dogs, cats). Kosher birds include domestic chicken, duck, geese, turkey, and pigeon and exclude predator and scavenger birds, with 24 non-kosher birds listed in the Torah. Reptiles, amphibians, worms, and insects are non-kosher. Kosher water creatures must have fins and scales (i.e. salmon, tuna, pike, flounder, carp, herring). Excluded are catfish, sturgeon, swordfish, lobster, shellfish, crabs, and water mammals.

What terms are used for "permitted" foods in Islam and Judaism? What terms are used regarding proper slaughter of animals?

Go to: <https://rmiq.eatrightpro.org/rmiq/subgroups/jewish/jewish-resources/traditional-kosher-rules> and answer the following questions:

1. What is pareve?
2. Is honey kosher?
3. Explain the separation of milk and meat.
4. What are the 9 rules from which the kashrut is derived?
5. Describe the process of ensuring that produce is kosher.

More details on Islam and Judaism on the following pages.

Islam

Before reading this section, please read this introduction to Islam by Dr. Elizabeth Macaulay and published at Khan Academy: <https://bit.ly/3NoxRPU>

- Review “Introduction to Islam”, “the Five Pillars of Islam”, and “Hajj”
- *What are the 5 pillars of Islam?*

Traditional Muslim food practices center around foods that are *halal* (permitted) or *haram* (harmful), as described in a previous section, and around holy days. Foods may be *haram* because of the food itself or the source of food, or because of how it was prepared (*Zabihah*). With relevance to nutrition counseling, the RDN should be aware of ingredients found in commercial or processed foods that are haram, including use of animal-source enzymes in cheese and yogurts, shortening, lard, and gelatin as ingredients, and others listed in Table 1 below.

Table 1. Haram foods

• Pork • Non-Zabihah animals or products (broth) • Alcohol and other intoxicants (tobacco) • Carnivorous animals (fox, wolf, dog, cat, etc) • Birds of prey (eagle, hawk, falcon, etc) • Land animals without external ears (snakes, etc) • Blood and blood byproducts (blood pudding)	• Enzymes from animal sources (pepsin, lipase) • Ingredients made with haram foods (vanilla extract made with alcohol, rum cake) • Whey from animal rennet • Animal shortening, lard, gelatin • Mono- and diglycerides from animal source • Sodium stearoyl lactylate (used in bread) • L-cysteine (used in dough and flour)
--	--

References: Guide to Understanding Halal Foods¹¹ and Cultural Food Practices by Diabetes Care and Education DPG.

Muslim holy days include:

- **Al-Hijra:** the Islamic New Year which marks the end of Mohammad’s journey from Mecca to Medina. Some Muslims celebrate this day with desserts such as fruit- and nut-filled pastries, cookies, and crepes.
- **Eid al-Adha:** the Feast of Sacrifice is a 3-4 day holiday marking the sacrifice of Abraham and celebrating the Hajj.¹² Muslims may celebrate with a symbolic sacrifice of a lamb, goat, cow, or another animal, that is then divided into 3 to be shared. The meat may be used to make kebabs, stews, roast, or other dishes including organ meats.
- **Ramadan:** Ramadan is a holy month of fasting, reflection, and prayer. During Ramadan, Muslims do not drink or eat from dawn until sunset and then break their fasts by sharing meals with friends and family.¹³ Individuals exempt from fasting are those who are sick, have conditions that complicate fasting (like diabetes), traveling, pregnant, breastfeeding, menstruating, children not of puberty age, and the elderly if their condition warrants exemption. Considerations for fasting should be taken, particularly diabetes-related complications of fasting like hypoglycemia and diabetic ketoacidosis. If a physician determines a person with diabetes is OK to fast for Ramadan, the RDN may work with them to individualize their meal plan, monitor their glucose, and determine appropriate nutrition when breaking the fast as well as appropriate but not excessive exercise.¹⁴
 - Please review: *Ramadan: The Practice of Fasting* by Sara Elnakib, MPH RD at <https://www.eatright.org/health/lifestyle/culture-and-traditions/ramadan--the-practice-of-fasting>
- **Eid-al-Fitr:** the Feast of Fast-Breaking signifies the end of Ramadan and lasts for 3 days. It emphasizes charity, gift-giving, special prayers, and time with family, and celebrating their accomplishment of completing Ramadan.¹⁵ Special meals typically feature sweet dishes (honey cake, Turkish delight, cookies, baklava).

Judaism

Before reading this section, please read this introduction to Judaism by Dr. Jessica Hammerman and Dr. Shaina Hammerman and published at Khan Academy:

<https://bit.ly/3uqCsIN> - Review "Introduction to Judaism" and answer these 3 questions

- *What texts make up the Tanakh?*
- *What is the Talmud?*
- *How is "Judaism a way of life that honors the cycle of days, weeks, months, years, and lives"?*

Using the resources from the Religion Member Interest Group

(<https://rmig.eatrightpro.org/rmig/subgroups/jewish/jewish-resources/traditional-kosher-rules>)

and at <https://www.jewfaq.org/kosher> to make and fill out a table like the one below.

Kosher	Non-Kosher	Pareve

Then, watch this video on kosher food preparation and guidelines (you may need to make an account on the website to watch the entire video): <https://study.com/academy/lesson/kosher-food-preparation-guidelines.html>

Jewish dietary practices may vary depending on how observant an individual is, and to what traditions they adhere. Some different cultural groups include Sephardic Jews, Ashkenazi Jews, Eastern European Jews, and Turkish Jews, among many others. One example of how things may differ is the special foods eaten during holy days. The information below is meant to be general. Get to know your clients by asking them about how they observe holy days and what their favorite recipes and dishes are.

- **Shabbat** is observed weekly on the Sabbath, with the goal of pausing busy lives to rest, focus on what is important, and connect with loved ones. Traditions include blessings over candles on Friday night, wine, and challah bread. Traditional meals vary by cultural group and may be modified with modern twists (gluten-free, vegetarian or vegan, incorporation of other cultural dishes).
- **Rosh Hashanah** is the Jewish new year, and in the West occurs during autumn time. The holiday celebrates discovery, introspection, and new beginnings. Sweet foods are used to represent wishes for a "sweet new year" and include apples dipped in honey and other sweet dishes. Challah bread is baked in a round shape to represent eternity of time and made extra sweet with raisins.
- **Yom Kippur** is the Day of Atonement and is a day of fasting and prayer. The fast is broken in the evening with a light dairy meal (bagels and fish, kugel, eggs, salads, blintzes).
- **Passover** is a holy day celebrating the exodus of Israelites from Egypt and occurs in the spring. The Passover Seder, the holiday meal, includes symbolic foods (look up images of Seder plates) and the abstaining from leavened foods (matzah instead of challah).
- Other Jewish holy days include Sukkot, Simchat Torah, Hanukkah, Tu b'Shevat, Purim, Shavuot, and Tisha b'Av. Tisha b'Av is a day of fasting, and some may keep a simple dairy-focused diet in the week before.

Cultural Dietary Food Practices

The following information is based on resources written by experienced nutrition counselors¹⁴ and qualitative research studies exploring the different cultural dietary food practices found in different groups living in the US. *However*, there is a large amount of diversity within a single culture, and the information and recommendations here are not meant to be a *one-size-fits-all* approach. The information in this packet is meant to serve as an introduction to different cultural dietary food practices. If you work as an RD who serves one of these communities, it is important to learn much more about that community's culture, traditions, and food practices, so that you can better serve your clients. If you find any of the wording or language in this document to be incorrect or offensive, please reach out to studysmartermethod@gmail.com so that we can make immediate corrections. Thank you.

With religious dietary practices we talked about a few general categories including abstaining from certain foods regularly or during holy days, fasting practices, and food preparation. With cultural dietary practices, there are both counseling and nutrition considerations. Counseling considerations include how a culture demonstrates courtesy and respect as well as typical nonverbal communication (i.e. is eye contact expected or avoided?). Nutrition considerations include traditional foods and food preparation methods that you should be aware of and work with your client to build healthy habits rooted in their existing cultural practices.

The following pages include information summarized from various resources that are linked at the beginning of specific sections, and from the Cultural Food Practices book by the Diabetes Practice group. That book goes in-depth to each culture and is a good resource for you to use in the future to learn more if you are working with specific communities.

For the RD exam, however, it is more important to think about how you would achieve the following bullet points for different cultures.

For all clients, it is important to:

- Demonstrate courtesy and respect
- Interpret nonverbal communication
- Inquire about clients' understanding of diseases and medications
- Discuss use of complementary and alternative medicine
- Consider family demands and dynamics

However, how you achieve the above bullet points may differ by culture. Greetings in some cultures include a firm handshake and eye contact, while other cultures typically avoid direct eye contact. Respect of elders is common in many cultures, so you should ask clients who are older than you how they would like you to address them, erring on the more formal side. For clients who are substantially younger than you, you may be treated not only with the respect of an elder, but they may also hesitate to open up to you. With all clients, it is important to build rapport. When working with a client from a culture different than yours, it may take more sessions to establish that rapport. One step you can take at the beginning of sessions is asking them to choose where they would like to sit, and asking if they prefer to sit next or near to them, or directly across from them.

Family is central to many cultures that you might encounter and this may manifest in several ways. Your client may or may not be the person in charge of food decisions or shopping in the house. If they are not, consider asking if they would want someone to accompany them to counseling sessions – they may bring someone who helps take care of them or the person who makes food decisions. Other clients may feel embarrassed including a family member in the session. It is important to ask questions in an open-ended, non-assuming way, so that they do not feel that they are answering “right” or “wrong”. Share with them that you want to get to know

them and make the counseling experience as comfortable as you can, and so you may be asking questions and they are always free to decline answering. In fact, having a conversation primarily composed of asking questions may be considered rude in some cultures, so it is important to preface your questions through explanation of why you are asking the question. For example, before asking them about their home environment, you would explain that you want to be able to make recommendations that will work for them, and so you would like to know what their kitchen looks like, who prepares the food, and how often they grocery shop.

Another important part of interacting with client from other cultures is recognizing that their distrust in health care providers and the healthcare system is valid. Many groups that have been marginalized historically and presently may be wary of opening up to a health care provider such as an RD. Taking time to emphasize that everything they say is kept confidential, and that your goal is to help them set individualized goals and help them achieve them, can help them feel more comfortable speaking with you and working towards the goals you set together.

Rather than administering a literacy test, use different methods of interaction and communication to determine what your client prefers. Using food models and pictures of food rather than a stack of handouts can help you both communicate better with one another, and also helps give client something to focus on if they are not comfortable with eye contact. Some clients may read English better than they speak it or may want to take a handout home because they have someone at home who speaks English. However, there are many educational handouts available that focus more on graphics than words. These are great tools to send home with the client as a reminder of what you discussed, or if necessary, to use as a visual to guide the session. You want to avoid presenting them with a text-heavy handout that you read from during the session.

Diabetes and kidney disease disproportionately affect non-White populations in the US. Before assuming what your client knows about the disease, they are there to see you for, or disease they may be at risk for, assess their knowledge by saying "Tell me what you know about diabetes". They may have family members who have these diseases and understand the consequences of the disease but may not understand what is going on in the body or how different medications work. Explaining these concepts in laymen's (non-medical jargon) terms is important and empowers your client. You can also use the opportunity to provide accurate information to correct any misconceptions they express, in a respectful way. You should also ask, non-judgmentally, about traditional medicine, supplements, herbs, and healing practices they are using. You can use laymen's terms to explain potential interactions between their medications and supplements they are taking, if necessary.

Work with clients to identify challenges and barriers to meeting their goals, which may include work schedules, inconsistent access to transportation, family commitments, or lack of control over food selection. Establishing rapport, encouraging positive change, and reassuring clients that your discussions are confidential will build trust between you and your client. You can help your client be more successful by learning about their environment, including their access to cooking equipment and where they like to grocery shop. You can make recommendations more individualized by providing examples and making plans with them that they can directly apply to their life.

The Nutrition Care Process of Assessment, Diagnosis, Implementation, Monitoring, and Evaluation is the same no matter your clients' culture. When working with a client from a culture different than yours, you must recognize that you are also a learner. Being respectful, reminding that you will maintain their confidentiality and privacy, explaining why you are asking them certain questions, as well as asking them about their preferences for how you address them or interact, are key steps in building rapport. The Assessment process for all clients involves understanding their home environment as it relates to food preparation and their use of

alternative medicines and healing practices. Through your own efforts of learning more about the communities you interact with, you may be more knowledgeable about some answers they give than others. If you don't know something, ask them more questions (or ask them to "tell you more"). By asking open-ended questions in non-judgmental tones, you will build a relationship where the client feels comfortable sharing information with you.

Nutrition education and interventions may include:

- Encouraging modification of traditional dishes with low-fat preparation methods or ingredients – *have the client bring in or describe their favorite recipe and then discuss how it could be modified*
- Discuss alternatives to traditional high-fat or high-sodium foods that they feel comfortable with
- Provide education on portion control with examples of meals they would commonly eat
- Discussing potential interactions between supplements and medications
- Address misconceptions about certain foods or diseases
- Encouraging forms of physical activity that works with their lifestyle and culture
- Discuss strategies to make healthy selections of foods available to them where they shop, including commodity foods through food programs
- Addressing barriers to achieving their goals, including family dynamics around food, and helping them to select healthy alternatives that are still feel culturally feasible

What strategies are listed on the previous pages that you can do to make clients feel more comfortable?

Resources to Review

American Indian

- Food and Culture fact Sheet at https://dune.une.edu/cgi/viewcontent.cgi?article=1007&context=an_studedres

Caribbean

- The Food and Culture of Haiti at https://dune.une.edu/cgi/viewcontent.cgi?article=1009&context=an_studedres
- Jamaican Food and Culture at https://dune.une.edu/cgi/viewcontent.cgi?article=1005&context=an_studedres

Filipino

- Fact Sheet: Filipino Food and Culture at https://dune.une.edu/cgi/viewcontent.cgi?article=1143&context=an_studedres
- Filipino Food, Dietary Customs, and Health Beliefs at https://dune.une.edu/cgi/viewcontent.cgi?article=1170&context=an_studedres

African American

- “What is Black Food?” by Anela Malik at <https://www.feedthemalik.com/post/what-is-black-food> ¹⁶
- “Cooking by Feeling: Honoring Black Culture through Food” by Alicia Wallace at <https://www.healthline.com/nutrition/black-cooking-heritage> ¹⁷
- “Healthy Soul Food, Your Way” by Marisa Moore, MBA RDN LD at <https://www.eatright.org/health/lifestyle/culture-and-traditions/healthy-soul-food-your-way>
- “Factors influencing food choices, dietary intake, and nutrition-related attitudes among African Americans: application of a culturally sensitive model” by Delores CS James at <https://bit.ly/367YyaY>

Hispanic/Latine

- Check out these Latine RDs featured: <https://www.verywellfit.com/latinx-dietitians-and-nutritionists-to-follow-5101227>
- “Cultural Diversity: Eating in America – Mexican-American” from the Ohio State University Extension at <https://bit.ly/3IHbGRJ>
- Mexican Food and Culture Fact Sheet at https://dune.une.edu/cgi/viewcontent.cgi?article=1010&context=an_studedres
- Food and Culture In Cuba: https://dune.une.edu/cgi/viewcontent.cgi?article=1008&context=an_studedres

Asian and Indian

- Culturally Informed Clinical Practices for Healthy Hmong Lifestyles at <https://bit.ly/3LeJqHQ>
- Nutrition and Fasting in Vietnamese Culture at <https://ethnomed.org/resource/nutrition-and-fasting-in-vietnamese-culture/>
- Fact Sheet: Vietnam Food Culture at https://dune.une.edu/cgi/viewcontent.cgi?article=1114&context=an_studedres
- Fact Sheet: Food and Culture in Japan at https://dune.une.edu/cgi/viewcontent.cgi?article=1143&context=an_studedres
- Food Culture of India: https://dune.une.edu/cgi/viewcontent.cgi?article=1174&context=an_studedres
- Fact Sheet: Indian Cuisine and Culture at https://dune.une.edu/cgi/viewcontent.cgi?article=1169&context=an_studedres
- Healthy Korean Food choices, according to a Dietitian at <https://www.healthline.com/nutrition/is-korean-food-healthy#korean-food-basics>

Cajun and Creole

- The Creole State: An Introduction to Louisiana Traditional Culture at https://www.louisianafolklife.org/LT/Articles_Essays/creole_art_creole_state.html
- What does it mean to be Cajun? 12 stories to understand this identity at <https://www.hnoc.org/publications/first-draft/what-does-it-mean-be-cajun-12-stories-understand-identity>
- Cajun or Creole: What’s the Difference? At <https://www.neworleans.com/restaurants/where-to-eat/cajun-or-creole/>

Religious Dietary Practices Worksheet

	Foods Not Eaten	Fasting Practices	Other Notes
Buddhism			
Catholicism			
Church of Latter-Day Saints (Mormon Church)			
Eastern Orthodox Christians			
Hinduism			
Islam			
Judaism			
Seventh Day Adventists			
Sikhism			

Cultural Dietary Food Practices Worksheet

Use the Cultural Food Practices book or online resources to complete this worksheet.

	Counseling Considerations	Nutrition Considerations
African American		
American Indian and Alaska Native		
Asian Indian and Pakistani		
Cajun and Creole		
Caribbean Hispanic		
Central American		
Chinese American		
Filipino American		
Hmong American		
Korean American		
Mexican American		
South American		

References

- ¹ Makela J. Cultural definitions of the meal. n: Meiselman HL, ed. Dimensions of the Meal: The Science, Culture, Business, and Art of Eating. Gaithersburg, MD: Aspen Publishers; 2000:7 – 18.
- ² Campinha-Bacote, Josepha. The Process of Cultural Competemility in the Delivery of Healthcare Service: Unremitting Encounters. 6th edition (2020). <http://transculturalcare.net/books-2/>
- ³ McCaffree, J. Dietary Restrictions of Other Religions. The Business of Dietetics. 2002. 102:7;912. [https://jandonline.org/article/S0002-8223\(02\)90212-9/fulltext](https://jandonline.org/article/S0002-8223(02)90212-9/fulltext)
- ⁴ Van De Walle, G. Buddhist Diet: How it Works and What to Eat. *Healthline*. April 14, 2020. <https://www.healthline.com/nutrition/buddhist-diet>
- ⁵ Stuckrath, T. Religious Dietary Restrictions: Your Essential Quick Reference Guide. Thrive! Meetings & Events. 24 Jan 2018. <https://thrivemeetings.com/2018/01/religious-dietary-restrictions-guide/> Accessed 2 June 2021.
- ⁶ For Rastas, Eating Pure Food from the Earth is a Sacred Duty. National Geographic. <https://www.nationalgeographic.com/culture/article/for-rastas--eating-from-the-earth-is-a-sacred-duty> Accessed 25 March 2022.
- ⁷ Ital. Wikipedia. <https://en.wikipedia.org/wiki/Ital> ; Accessed 25 March 2022.
- ⁸ Fasting and feasting: A catholic perspective on the importance of Fasting. The faith & Belief Forum. <https://faithbeliefforum.org/fasting-and-feasting-a-catholic-perspective-on-the-importance-of-fasting/> Accessed 25 March 2022
- ⁹ Matalas AL, Turlouki E, and Lazarou C. Fasting and Food Habits in the Eastern Orthodox Church. <https://columbia.universitypressscholarship.com/view/10.7312/columbia/9780231149976.001.0001/upso-9780231149976-chapter-9> Accessed 25 March 2022
- ¹⁰ Traditional Kosher Rules. Religion Member Interest Group. Academy of Nutrition and Dietetics. <https://rmig.eatrightpro.org/rmig/subgroups/jewish/jewish-resources/traditional-kosher-rules> Accessed 25 March 2022.
- ¹¹ Guide to Understanding Halal Foods. *Toronto Public Health*. 2004. <https://www.halalrc.org/images/Research%20Material/Literature/Guide%20to%20Halal%20Foods.pdf> Accessed 15 March 2022.
- ¹² Eid al-Adha. Updated 21 Jul 2021. <https://www.history.com/topics/holidays/eid-al-adha> Accessed 15 Mar 2022.
- ¹³ Ramadan. Updated 14 Mar 2022. <https://www.history.com/topics/holidays/ramadan> Accessed 15 Mar 2022.
- ¹⁴ Diabetes Care and Education Dietetic Practice Group. Goody M and Drago L (editors). Cultural Food Practices. 2010.
- ¹⁵ Eid al-Fitr. History. <https://www.history.com/topics/holidays/eid-al-fitr> Accessed 25 Mar 2022.
- ¹⁶ Malik, Anela. What is Black Food? *Feed the Malik*. <https://www.feedthemalik.com/post/what-is-black-food> Accessed 18 Mar 2022.
- ¹⁷ Wallace, Alicia. Cooking by Feeling: Honoring Black Culture Through Food. *Healthline*. <https://www.neworleans.com/restaurants/wherhttps%3a/www.healthline.com/nutrition/black-cooking-heritage/>; accessed 25 march 2022